

Booking, Referrals, and Appointment Procedures

Scheduling rules, urgency categories, prerequisites, and failed-contact workflow

Document ID	Version	Effective	Review due
NB-OPS-301	1.0	2026-08-01	2027-08-01

FICTIONAL TRAINING DOCUMENT - SYNTHETIC DATA ONLY. Not a real healthcare provider, not clinical guidance, and not for patient care. All people and contact details are invented.

Owner: Practice Operations Manager
Approved by: Fictional Clinical Governance Committee
Classification: Training / public synthetic corpus

Document control and RAG metadata

Field	Value
Canonical title	Booking, Referrals, and Appointment Procedures
Document ID	NB-OPS-301
Keywords	booking, appointment, referral, follow-up, cancellation, no-show, urgent
Authority	Practice Operations Manager
Supersedes	None - initial synthetic edition
Applicability	Training simulations at Northbridge Cardiology Practice only

Retrieval rule

Retrieve whenever the output includes tests, future appointments, referrals, result review, or contact attempts.

1. Booking channels and fictional contacts

Service	Contact	Hours
Routine cardiology booking	01632 960 110	Mon-Fri 08:30-17:00
Diagnostics booking	01632 960 120	Mon-Fri 08:00-18:00
Medication review desk	01632 960 130	Mon-Fri 09:00-16:30
Same-day cardiology triage	01632 960 199	Mon-Fri 08:00-18:00
Out-of-hours fictional clinical line	01632 960 999	Outside practice hours; training-only number
Accessible booking email	appointments@northbridge.example.invalid	Not monitored for emergencies

2. Urgency categories

Category	Target	Examples of workflow triggers	Booking rule
Emergency	Immediate emergency pathway	Severe chest pain, severe breathlessness, collapse, stroke-like signs, uncontrolled major bleeding	Do not book routine care; display emergency instruction and stop normal workflow
Same day	Clinical triage today	Significant new/worsening symptoms, urgent abnormal result, medication safety conflict	Warm transfer to triage; named clinician owns outcome
Urgent	Within 2 working days	Clinically important but stable deterioration or time-sensitive medication review	Book protected urgent slot and required prerequisite tests
Soon	Within 2 weeks	Post-initiation review, early adverse-effect review, noncritical abnormal test	Offer appropriate modality; attach review purpose
Routine	Within 6-12 weeks or signed target	Stable follow-up and planned surveillance	Book against clinician-specified window
Administrative	Within 5 working days	Record copy, nonclinical letter, rescheduling	Operations queue; no clinical interpretation

3. Minimum booking dataset

- Patient ID and verified identifiers; preferred contact and accessibility needs.
- Appointment/test type, clinical purpose, urgency, requested window, and ordering clinician.
- Prerequisites such as labs, medication list, fasting status only if ordered, transport/support needs, and interpreter.
- Results owner and planned review mechanism.

- For remote review, confirm location, privacy, technology, and contingency contact.

4. Cancellation and non-attendance

- Reschedule within the original clinical window where possible.
- A safety-critical missed appointment is returned to the clinical owner the same day; booking staff do not decide whether it can wait.
- Document contact attempts without stigmatizing language.
- After two failed contacts, verify contact details and ask clinician to define the next safe action.
- Never cancel ongoing medicine supply or discharge automatically because of non-attendance.

5. Closed-loop follow-up

Every recommendation for a test or appointment must name: what, why, target date/window, booking owner, result/review owner, prerequisites, escalation if delayed, and how the patient will receive the outcome.

FICTIONAL TRAINING DOCUMENT - SYNTHETIC DATA ONLY. Not a real healthcare provider, not clinical guidance, and not for patient care. All people and contact details are invented.